

After your extraction

RECOVERY GUIDE · ROUTINE, SURGICAL, & BONE GRAFT

Keep this guide handy
Call our office with any questions

Your tooth has been removed. Sometimes we also place a bone graft to preserve the shape of your jaw for a future implant, bridge, or denture. Most of the healing will happen on its own — your main job in the first few days is to protect the blood clot and give the tissue time to close.

TODAY	DAYS 1-3	DAYS 4-7	WEEK 2+
Rest. Gauze. Ice. Soft, cool foods. Don't disturb the site.	Swelling peaks. Start saltwater rinses tomorrow. Pain tapers.	Most discomfort gone. Diet expands. Watch for dry socket.	Tissue closes over. If you had a graft: full bone healing takes 3-4 months.

1 The first 24 hours (most important)

PROTECT THE CLOT

A soft, Jell-O-like blood clot forms in the socket. It's your body's natural bandage — losing it early is the main cause of "dry socket."

- ✗ No rinsing, swishing, or spitting for 24 hours — let saliva gently drain
- ✗ No straws, no vaping, no sucking motions for 72 hours
- ✗ No smoking for at least 72 hours — smoking roughly triples the risk of dry socket
- ✗ Don't poke the site with your tongue, finger, or toothpick

CONTROL BLEEDING

Bite firmly on the gauze we gave you for 30-45 minutes. Some oozing and pink-tinged saliva for the first day is normal.

 **If bleeding restarts:** fold a fresh gauze pad (or a damp black tea bag — the tannins help clotting), place it directly on the socket, and bite firmly without checking for 30-45 minutes. Keep your head elevated.

ICE FOR SWELLING

- 20 minutes on, 20 minutes off, for the first 24-48 hours
- Peak swelling is at 48-72 hours — not the day of surgery. This is normal.
- From day 3, switch to warm moist compresses

EAT SOFT, COOL FOODS

GOOD CHOICES

- ✓ Yogurt, cottage cheese, pudding
- ✓ Smoothies (spoon, no straw)
- ✓ Mashed potatoes, applesauce
- ✓ Scrambled eggs, soft pasta
- ✓ Lukewarm soup, broth

AVOID FOR THE FIRST FEW DAYS

- ✗ Hot or spicy foods for 72 hours (can restart bleeding)
- ✗ Hard, crunchy, chewy, or sharp foods
- ✗ Chips, nuts, popcorn, crusty bread
- ✗ Carbonated and alcoholic drinks
- ✗ Chewing directly on the surgical side

REST

Take the rest of the day off work or school. No exercise, heavy lifting, or strenuous activity for 5 days — increased blood pressure can restart bleeding. Sleep with your head slightly elevated (an extra pillow) the first night.

2 Pain & swelling

Discomfort usually peaks at 48–72 hours, then tapers quickly. A combination of **ibuprofen + acetaminophen** is as effective as prescription opioids for most dental pain — and safer. See your **Medication Guide** for exact dosing.

- Start your pain medication *before* the numbness wears off
- Take ibuprofen with food to protect your stomach
- Stay ahead of pain — use scheduled doses for the first 48–72 hours rather than waiting
- If an opioid was prescribed, only take it if ibuprofen + acetaminophen isn't enough

Days 2 through 7 and beyond

EXTRACTION RECOVERY · CONTINUED

3 Days 2 through 7

START SALTWATER RINSES TOMORROW

Mix ½ teaspoon of salt in 8 oz of warm water. Rinse gently 3–4 times a day, especially after meals. **Let the water fall out of your mouth** rather than forcefully spitting. Continue for 1–2 weeks.

i If you were prescribed a **chlorhexidine rinse** (Peridex, Paradex, Periogard), use it twice daily as directed, and use saltwater the rest of the day. Avoid eating or drinking for 30 minutes after rinsing.

ORAL HYGIENE

- Brush the rest of your teeth normally starting the day after surgery
- Avoid the extraction site directly for the first 3 days
- After day 3, a soft toothbrush can gently clean near (but not in) the site
- Keep flossing everywhere except right next to the surgical area

WHAT'S NORMAL VS. WHAT'S NOT

NORMAL (EXPECTED)

- ✓ Swelling peaking at 48–72h, then improving
- ✓ Mild bruising of cheek or jaw
- ✓ Yellow or whitish tissue in the socket (healing, not pus)
- ✓ Slight bad taste or smell for a few days
- ✓ Stiff jaw that slowly improves
- ✓ Sensitivity in neighboring teeth
- ✓ Slightly altered or "less sharp" sensation in lip, chin, or tongue that's gradually improving

CALL OUR OFFICE

- ✗ Severe pain on day 3–5 that medication doesn't touch at all (possible dry socket)
- ✗ Pain, swelling, or bad taste that returns *after* you'd started to feel better (possible infection)
- ✗ Bleeding that won't stop after 30–45 min of firm pressure
- ✗ Fever over 101°F
- ✗ Numbness or tingling in lip, chin, or tongue that isn't improving after 2 days

💡 Is it dry socket? Here's a simple test. If your pain medication brings pain down to a manageable level (around **2 or 3 out of 10**), it's *not* dry socket — you're on a normal healing curve. If the medication does **nothing at all** — the pain stays severe no matter what you take — that's the hallmark of dry socket. Call us; it's easily treated with a medicated dressing that brings fast relief.

💡 **Is it an infection?** Watch the *direction* of your pain, not just how much it hurts. Days 2–4 are usually the peak. After that, each day should be a little better than the last. If you were feeling better and then the pain, swelling, or a bad taste suddenly comes back, that's the classic pattern for an infection. Call us.

i Why do these happen? Dry socket and infection are the most common complications we see — about **5–12% of cases combined** — and both are usually straightforward to treat. The mouth isn't sterile the way skin is. Unlike a cut on your arm, we can't scrub the site clean of bacteria before surgery — they're everywhere in the mouth. Even with sterile instruments and careful technique, some patients still develop one. The risk is higher if the tooth was already infected before we removed it.

i About nerve sensation: It's common for the lip, chin, or tongue to feel a little "off" or less sharp for a day or two after some extractions — this is usually from the nerve being bruised, and it almost always fully recovers on its own. What we want to know about is **complete numbness** (the feeling of still being frozen from dental anesthetic) that lasts more than 2 days.

SUTURES

If dissolvable sutures were placed, they'll fall out on their own over 7–14 days. If non-dissolvable sutures were placed, you'll come back for removal — typically 7–10 days after your procedure.

ONLY IF YOU HAD A BONE GRAFT

Your bone graft

A graft was placed to preserve the bone shape for a future implant, bridge, or denture fit. You'll also receive the **Bone Graft Recovery Guide**, and where it is stricter, it wins: **no tobacco or vaping for 1 month** and **no alcohol for 7 days** (not 72 hours), and Peridex is a **1-minute soak** for 1 week after surgery, then a swab for 2 more weeks. A few things to expect:

- You may see small whitish-gray particles — this is graft material. **Do not pick at them.**
- A few grain-like bits in your saliva over the first few days is normal. Some loss is expected — you don't need to save them.
- **Don't probe with your tongue.** Pressure can shift the graft before it integrates.
- Sometimes the membrane comes untucked or even comes all the way out. Please give us a call if it does.
- Full bone healing takes **3–4 months** before the site is ready for an implant or other restoration.
- If you wear a denture or partial that covers this area, **don't wear it until we adjust it** — pressure can disturb the graft. (Exception: an **immediate denture** placed today stays in for the first 24 hours.)

ONLY IF THE BONE WAS SMOOTHED (ALVEOPLASTY)

Bone smoothing (alveoplasty)

Alongside your extraction, we reshaped the bone underneath — smoothing sharp edges or high points — so the area will be comfortable and ready for a future denture, partial, or implant. This adds a little to your recovery:

- Expect **slightly more swelling and soreness** than a routine extraction for the first 2–3 days.
- You may feel a small, sharp bony edge under the gum as it heals — usually this smooths out on its own as tissue and bone remodel over several weeks.
- If a small sharp piece (a "bone spicule") works its way through the gum weeks later, don't worry — it's a normal part of healing. Call us and we'll remove it quickly if it bothers you.
- Full bone remodeling takes 2–3 months.

5 Other things that may help

A few evidence-backed additions that support recovery:

- **Bromelain** (500 mg, 3× daily, 5–7 days) — a pineapple-derived enzyme with research support for reducing swelling and speeding recovery after oral surgery. *Skip if you take a blood thinner, have a bleeding disorder, or are allergic to pineapple.*
- **Sleep** — 7+ hours a night the first week. Sleep loss measurably slows healing.
- **Protein** — aim for 80–100g daily. Soft options: Greek yogurt, cottage cheese, eggs, protein smoothies (spoon only), soft fish.
- **Hydration** — plenty of fluids (water or an electrolyte drink). Not alcohol.
- **No nicotine** — smoking, vaping, and nicotine pouches all slow healing and raise the risk of dry socket. The longer you can stay off, the better.
- **No alcohol for at least 72 hours** — interacts with medications and slows healing.

i Supplements not on this list (arnica, megadose vitamin C, "immune boosters," CBD) don't have strong evidence for healing after oral surgery. Save your money.

When to call us

- Severe pain on day 3–5 that your medication doesn't help at all (possible dry socket)
- Pain, swelling, or bad taste that returns after you'd started to feel better (possible infection)
- Heavy bleeding that won't stop after 30–45 min of firm gauze pressure
- Swelling spreading to your neck or around your eye
- Fever over 101°F
- Numbness or tingling in lip, chin, or tongue that isn't improving after 2 days
- Jaw stiffness getting worse (not better) after day 5

URGENT — DON'T WAIT

- Difficulty breathing or swallowing
- Rapidly spreading swelling to your neck, under your tongue, or around your eye

Call our office right away for the on-call provider. If you truly cannot breathe or swallow, call 911.

Most calls are easy to resolve. If you're unsure whether something is normal, call us — we'd rather answer than have you worry about it.

Acknowledgment & signature

I acknowledge that I have received and understand these instructions, and all my questions have been answered. I agree to follow them and to call the office with any questions or concerns.

PATIENT SIGNATURE

DATE

PATIENT PRINTED NAME

DATE OF BIRTH

PARENT / LEGAL GUARDIAN / CAREGIVER SIGNATURE (IF SIGNING FOR THE PATIENT)

DATE

PRINTED NAME

RELATIONSHIP TO PATIENT